



American Society of
Anesthesiologists®

American Society of Anesthesiologists Statement on ASA Physical Status Classification System

ASA Committee of Oversight: Economics

Approved by the ASA House of Delegates on October 15, 2014, and last amended on October 15, 2025

The American Society of Anesthesiologists (ASA) Physical Status Classification System has been in use continuously since 1961. The purpose of the system is to assess and communicate a patient's pre-anesthesia medical comorbidities. The classification system in conjunction with other factors (*e.g.*, type of surgery, frailty, level of deconditioning) helps anesthesiologists estimate perioperative risk. Patients with ASA Physical Status III or higher generally require more extensive pre-anesthesia evaluation and coordinated care along with more intense anesthesia care as compared with patients having ASA Physical Status I or II.

The definitions and examples shown in the table below are guidelines for the clinician. To improve communication and assessments at a specific institution, anesthesiology departments may choose to develop institutional-specific examples to supplement the ASA-approved examples.

Assigning a Physical Status classification level is a clinical decision based on multiple factors. While the Physical Status classification may initially be determined at various times during the preoperative assessment of the patient, the final assignment of Physical Status classification is made on the day of anesthesia care by the anesthesiologist after evaluating the patient.

Current Definitions and ASA-approved Examples

ASA PS Classification	Definition	Adult Examples, Including, but not Limited to:	Pediatric Examples, Including but not Limited to:	Obstetric Examples, Including but not Limited to:
ASA I	A normal healthy patient	Healthy, non-smoking, no or minimal alcohol use	Healthy (no acute or chronic disease), normal BMI percentile for age	
ASA II	A patient with mild systemic disease	Mild diseases only without substantive functional limitations or end organ involvement. Current smoker, social alcohol drinker, pregnancy, obesity (30<BMI<40), well-controlled DM or HTN, mild lung disease, CHF NYHA Class 1, mild cognitive dysfunction, isolated mild/moderate obstructive sleep apnea with compliance with CPAP as prescribed.	Asymptomatic non cyanotic congenital cardiac disease, well controlled dysrhythmias, asthma without exacerbation, well controlled epilepsy, non-insulin dependent DM, abnormal BMI percentile for age, mild/moderate OSA, oncologic state in remission, autism with mild limitations, malignant hyperthermia risk (family history or patient with history)	Normal pregnancy,* well controlled gestational HTN, diet-controlled gestational DM

(Continued)

Each ASA Statement is reviewed and approved by the ASA House of Delegates at least every 5 years. This Statement was approved by the 2025 ASA House of Delegates and appears here with slight modifications to meet journal style. ASA Statements are not evaluated by the Editors of ANESTHESIOLOGY OPEN.

Copyright © 2025 American Society of Anesthesiologists. This is an open-access article distributed under the terms of the Creative Commons Attribution-Non Commercial-No Derivatives License 4.0 (CCBY-NC-ND), where it is permissible to download and share the work provided it is properly cited. The work cannot be changed in any way or used commercially without permission from the journal. ANESTHESIOLOGY OPEN 2025:e0002. DOI: 10.1097/ao9.000000000000002

The article processing charge was funded by the American Society of Anesthesiologists.

ASA PS Classification	Definition	Adult Examples, Including, but not Limited to:	Pediatric Examples, Including but not Limited to:	Obstetric Examples, Including but not Limited to:
ASA III	A patient with severe systemic disease	Substantive functional limitations; One or more moderate to severe diseases. COPD, morbid obesity (BMI ≥40), active hepatitis, compensated cirrhosis, alcohol dependence or abuse, functional implanted pacemaker, moderate reduction of ejection fraction or CHF NYHA class 2 or 3, ESRD undergoing regularly scheduled dialysis, history (>3 months) of MI, CVA, TIA, PE, or CAD/stents, significant cognitive dysfunction, isolated severe OSA regardless of CPAP compliance or any severity of obstructive sleep apnea with CPAP noncompliance. Poorly controlled DM or HTN with or without end organ dysfunction.	Uncorrected stable congenital cardiac abnormality, asthma with exacerbation, poorly controlled epilepsy, insulin dependent diabetes mellitus, morbid obesity, malnutrition, severe OSA, oncologic state, renal failure, muscular dystrophy, cystic fibrosis, history of organ transplantation, brain/spinal cord malformation, symptomatic hydrocephalus, premature infant PCA <60 weeks, autism with severe limitations, metabolic disease, difficult airway, long term parenteral nutrition. Full term infants <6 weeks of age.	Preeclampsia with or without severe features, gestational diabetes poorly controlled or with complications or high insulin requirements, a thrombophilic disease requiring anticoagulation.
ASA IV	A patient with severe systemic disease that is a constant threat to life	Recent (<3 months) MI, CVA, TIA or CAD/stents, ongoing cardiac ischemia or severe valve dysfunction, severe reduction of ejection fraction or CHF NYHA class 4, shock, sepsis, DIC, ARDS, ESRD not undergoing regularly scheduled dialysis, uncompensated cirrhosis, severe cognitive dysfunction	Symptomatic congenital cardiac abnormality, congestive heart failure, active sequelae of prematurity, acute hypoxic-ischemic encephalopathy, shock, sepsis, disseminated intravascular coagulation, automatic implantable cardioverter-defibrillator, ventilator dependence, endocrinopathy, severe trauma, severe respiratory distress, advanced oncologic state.	Preeclampsia with severe features complicated by HELLP or other adverse event, peripartum cardiomyopathy, uncorrected/decompensated heart disease.
ASA V	A moribund patient who is not expected to survive without the operation	Ruptured abdominal or thoracic aneurysm, massive trauma, intracranial bleed with mass effect, ischemic bowel in the face of significant cardiac pathology or multiple organ/system dysfunction	Massive trauma, intracranial hemorrhage with mass effect, patient requiring ECMO, respiratory failure or arrest, malignant hypertension with hypertensive crisis, decompensated congestive heart failure, hepatic encephalopathy, ischemic bowel or multiple organ/system dysfunction.	Uterine rupture, amniotic fluid embolism
ASA VI	A declared brain-dead patient whose organs are being removed for donor purposes			

*Although pregnancy is not a disease, the parturient's physiologic state is significantly altered from when the woman is not pregnant, hence the assignment of ASA PS II for a woman with uncomplicated pregnancy.

**The addition of "E" denotes Emergency surgery: (An emergency is defined as existing when delay in treatment of the patient would lead to a significant increase in the threat to life or body part).

ARDS, acute respiratory distress syndrome; ASA, American Society of Anesthesiologists; BMI, body mass index; CAD, coronary artery disease; CHF NYHA, congestive heart failure, New York Heart Association classification; COPD, chronic obstructive pulmonary disease; CPAP, continuous positive airway pressure; CVA, cerebrovascular accident; DIC, disseminated intravascular coagulation; DM, diabetes mellitus; ECMO, extracorporeal membrane oxygenation; ESRD, end-stage renal disease; HELLP, hemolysis, elevated liver enzymes, low platelets; HTN, hypertension; MI, myocardial infarction; OSA, obstructive sleep apnea; PCA, post-conceptual age; PE, pulmonary embolism; PS, physical status; TIA, transient ischemic attack

References

For more information on the ASA Physical Status Classification system and the use of examples, the following publications are helpful. Additionally, in the reference section of each of the articles, one can find additional publications on this topic.

1. Abouleish AE, Leib ML, Cohen NH: ASA provides examples to each ASA Physical Status Class. *ASA Monitor* 2015; 79:38–49. doi:10.1097/01.asm.0001073116.40041.ee.
2. Hurwitz EE, Simon M, Vinta SR, et al.: Adding examples to the ASA-Physical Status Classification improves

- correct assignment to patients. *ANESTHESIOLOGY* 2017; 126:614–22. doi:10.1097/ALN.0000000000001541
3. Mayhew D, Mendonca V, Murthy BVS: A review of ASA Physical Status – historical perspectives and modern developments. *Anaesthesia* 2019; 74:373–9. doi:10.1111/anae.14569
 4. Leahy I, Berry JG, Johnson CJ, Crofton C, Staffa SJ, Ferrari L: Does the current American Society of Anesthesiologists Physical Status Classification represent the chronic disease burden in children undergoing general anesthesia? *Anesth Analg* 2019; 129:1175–80. doi:10.1213/ANE.0000000000003911
 5. Ferrari LR, Leahy I, Staffa SJ, et al.: One size does not fit all: a perspective on the American Society of Anesthesiologists Physical Status Classification for pediatric patients. *Anesth Analg* 2020; 130:1685–92. doi:10.1213/ANE.0000000000004277
 6. Ferrari L, Leahy I, Staffa SJ, Berry JG: The pediatric-specific American Society of Anesthesiologists Physical Status Score: A multi-center study. *Anesth Analg* 2021; 132:807–17. doi:10.1213/ANE.0000000000005025
 7. Horvath B, Kloesel B, Todd MM, Cole DJ, Prielipp RC: The evolution, current value, and future of the American Society of Anesthesiologists Physical Status Classification System. *ANESTHESIOLOGY* 2021; 135:904–19. doi:10.1097/ALN.0000000000003947